

Clinical Safety Report - Interim Analysis

Data Monitoring Committee Report #3

Date: March 15, 2025

1. Summary

This interim safety report summarizes adverse events reported during the first 12 weeks of the Phase 3 study of xanomeline-trospium in Alzheimer's disease. As of the data cutoff date, 450 patients have been enrolled and randomized (225 active, 225 placebo). The independent Data Monitoring Committee has reviewed all available safety data.

2. Adverse Events Overview

A total of 312 adverse events (AEs) were reported: 198 in the active treatment group and 114 in the placebo group. The majority of AEs were mild to moderate in severity. The most frequently reported AEs in the active group were nausea (22.7%), vomiting (14.2%), diarrhea (11.1%), and dizziness (8.9%). Five serious adverse events (SAEs) were reported: 3 in the active group (one hospitalization for pneumonia, one fall with hip fracture, one case of syncope) and 2 in the placebo group (one hospitalization for CHF exacerbation, one case of atrial fibrillation).

3. Laboratory Findings

No clinically significant differences were observed between treatment groups in hematology or clinical chemistry parameters. Transient elevations in liver transaminases (ALT/AST > 3x ULN) were observed in 4 patients (1.8%) in the active group and 2 patients (0.9%) in the placebo group. All elevations resolved without intervention. No Hy's Law cases were identified.

4. Vital Signs and ECG

Mean changes in systolic blood pressure were +2.1 mmHg (active) vs. -0.5 mmHg (placebo). No clinically meaningful changes in heart rate, respiratory rate, or body temperature were observed. ECG evaluations showed no evidence of QTc prolongation or other clinically significant arrhythmogenic signals in either treatment group.

5. DMC Recommendation

Based on the available safety data, the Data Monitoring Committee recommends that the study continue without modification. The safety profile is consistent with the known pharmacology of the study medications, and no unexpected safety signals have been identified. The risk-benefit assessment remains favorable for continued enrollment.